

The campus

Every room, and what the building can actually carry.

01 / FOUR STRUCTURES

711 sqm gross, 519 sqm of usable rooms.

Structure	Gross sqm	Rooms sqm	Efficiency	Role
Main building, ground	286	182	64%	Arrival, diagnostics and imaging, procedure, consulting
Main building, first	277	239	86%	The private medical plaza
Guest chalet	96	61	63%	Medbury Diagnostics laboratory
Boys' quarters	52	37	72%	Medbury Pharmaceuticals
Campus	711	519	73%	

The difference between gross and rooms is walls, circulation, the staircase and the verandas. 73% is normal for a converted residential building and it is the number to plan on, not the 711.

Areas are measured from the as-built PDFs. **Confirm against the CAD before anything is committed**, because every rate, capacity and cost figure in the whole document set re-bases if they are wrong.

02 / GROUND FLOOR

286 sqm gross. Arrival, diagnostics, procedure.

As built	Use	sqm	Note
Living Room	Reception, waiting and concierge	58.1	Single arrival point for the whole campus. 16 to 18 seats at premium spacing
Bedroom	Clean procedure room and recovery bay	24.7	Level trolley access. The only space where sedation can be done
Bedroom	Consulting room	19.3	General campus use
Kitchen	Phlebotomy and specimen handling	18.2	Existing water and drainage
Bedroom	Digital X-ray, lead shielded	14.1	Ground floor is mandatory: weight, shielding, no lift
Dining	Ultrasound and echocardiography	13.0	Serves the plaza's cardiology and orthopaedic users
Bedroom	Counselling and small office	10.4	Too small for consulting at premium standard
Bath	Imaging reporting and control	7.8	
Store	Medication collection point	6.0	Pharmacy's patient-facing presence
Bath x2, V.T	Patient WCs	10.4	Emergency pull cords required
		182.1	

03 / FIRST FLOOR

277 sqm gross. The private medical plaza.

As built	Use	sqm	Note
Living Room	Consulting 1, 2 and 3, plus spine	51.6	New partitions. Three rooms of about 17 sqm off a central corridor
Living Room	Doctors' and members' lounge	34.0	At the head of the stairs. Part of what the membership buys
Bedroom	FUE hair transplant suite	24.3	All-day single-patient cases. Relieves the reception rather than loading it
Playroom	Consulting room 6	22.4	Was the MDT room. Returns to consulting with the conversion clinic paused
Bedroom	Consulting room 4	19.8	En-suite WC
Bedroom	Consulting room 5	19.8	En-suite WC

Kitchen	Treatment room	14.8	Local anaesthesia only. Existing water and drainage
Bedroom	Consulting room 7	13.7	Was the coordinator room
Store	Linen and consumables	9.9	
Bath	Sterilising	9.4	Existing water and drainage
Bath	Patient WC	7.1	
Balcony	Terrace, off the lounge	6.2	Amenity for members
Bath	Phlebotomy draw point	5.7	So first-floor patients do not go downstairs for bloods
		238.8	

04 / GUEST CHALET**96 sqm gross. Medbury Diagnostics laboratory.**

As built	Use	sqm	Note
Bed Room	Laboratory, main	16.2	Benching and analysers
Living Room	Reporting office	11.8	
Kitchen	Laboratory, wet area	11.5	Existing water and drainage
Bed Room	Specimen reception and sorting	11.4	Receives from both phlebotomy points in the main building
Bath	Reagent cold chain and store	5.4	
Bath	Staff WC	4.3	
		60.7	

The laboratory fills the chalet. There is no room left in it for anything else, which is why Medlyfe cannot also be housed there and needs a separate decision.

05 / BOYS' QUARTERS**52 sqm gross. Medbury Pharmaceuticals.**

As built	Use	sqm	Note
Bed Room	Dispensary and stock	16.9	Dispensing bench and shelving
Bed Room	Product and skincare retail	14.9	Retail margin runs 40% to 60% and is a real line, not an afterthought
Bath	Controlled drugs store	2.9	Secure, with its own access control
Bath	Cold chain	2.7	
		37.4	

Patients never come here. They collect from the medication point in the main building, which is the pharmacy's only patient-facing space. Dispensing, stock and cold chain all sit out of the way in a separate structure, which is the right arrangement for a campus this size.

06 / WHAT THE CAMPUS CAN DO**Capacity, at stabilisation.**

Capacity	Number	Basis
Consulting rooms	8	7 on the first floor, 1 on the ground, all at full clinical standard
Talking-consultation room	1	The 10.4 sqm ground floor room. Psychology, dietetics, counselling. No examination couch
FUE hair transplant suite	1	One all-day case, so one arrival a day
Procedure rooms	2	Treatment room upstairs at local only, clean procedure room downstairs with sedation
Imaging modalities	3	Digital X-ray, ultrasound, echocardiography
Phlebotomy points	2	One per floor

Sessional blocks a year	8,832	8 rooms, 4 bands a day, 6 days, 46 weeks
Blocks sold at 41% fill	3,621	Fill varies by band, 35% to 55%
Consultations a year	18,105	At 5 patients a block
Arrivals a day	about 65	Including procedure and FUE

About 65 arrivals a day against a single-reception ceiling of roughly 98. That is real headroom, but not a great deal, and it assumes the two calendar rules hold: staggered start times in the early band, and timed arrival slots whenever the ground floor is running at volume.

07 / WHAT CAPS IT

Four constraints, in order of how hard they bind.

	Constraint	What it means
1	One reception	Seven to eight arrivals an hour sustained across the 14-hour window. This is the binding constraint on the whole campus and no amount of extra room capacity relieves it
2	No lift	No sedation-dependent work above ground level, at any budget. A sedated patient must be evacuated horizontally on a trolley
3	Waiting area	58 sqm carries 16 to 18 seats at premium spacing. Two waiting zones within it, so plaza patients and any high-volume clinic do not share seating
4	Ground floor imaging	X-ray cannot move upstairs. Weight, lead shielding and evacuation all fix it to the ground floor, which constrains everything else placed there

The building supports an eight-room plaza with proper diagnostics, two procedure rooms and a hair transplant suite, and that is a serious facility. What it will not support is a theatre, an inpatient bed, or a second high-volume clinic sharing the same front door. Those need either a lift, a modular unit on the grounds, or a different building.

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Room areas are measured from the as-built PDFs supplied and are approximate. Confirm against the CAD. Fill and patients-per-block assumptions are carried from the Ikoyi model and remain unvalidated for Abuja. Not a binding offer.